CMS TEAM Impact Report

Financial field guide for CMS's first mandatory **multi-procedure** surgical episode model · **721** hospitals · **107** in California (11 metro areas)

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Background

By 2030, every Baby Boomer will be 65 or older, with nearly **71,183** thousand Americans aged 65+ (1). Adults 65+ account for about **36.5%** of inpatient surgical stays with an OR procedure (2) — not 40% of all discharges.

Multimorbidity affects roughly 74% of older emergency general surgery patients in a **UK cohort** (3). Frailty and dementia sharply raise surgical mortality (4–6).

The AAMC projects a shortage of up to **86,000** physicians by **2036**, including up to **19,900** surgeons (7). Average THA/TKA length of stay fell from **3.0** to **1.1** days (2012–2023) (8); more than half of Medicare TKAs are billed outpatient (9).

TEAM holds hospitals accountable for **30-day** episodes (10). **CJR-X** starts **January 1, 2028**, covering **2,217** hospitals (11). About **2,938** hospitals will sit in mandatory episode models by that date.

> **Note**
>
> **Stat box (computed).** TEAM hospitals: **721** · CJR-X: **2,217** · 65+ OR-stay share: **36.5%** · Surgery prevalence 65+: **19.4%** (12)

Demographic wave

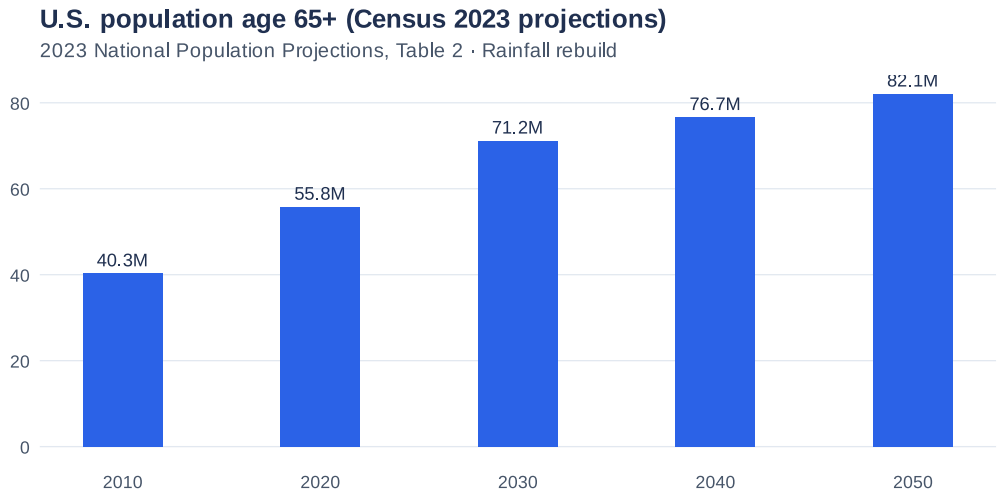


Figure 1: Figure 1. U.S. population age 65+ (Census 2023 projections).

Surgical demand in older adults

Nearly **1 in 5** adults 65+ had surgery in the past year — the highest prevalence of any age group (12).

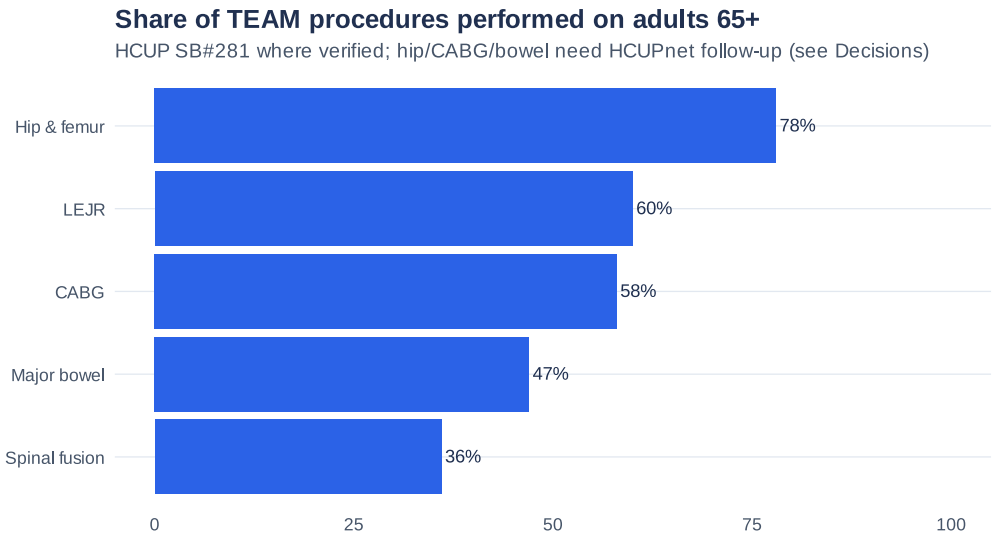


Figure 2: Figure 2. Age 65+ share by TEAM procedure (HCUP where verified).

TEAM mandate

721 mandated hospitals · CMS savings target **$481M** over five performance years (10). Up to **20%** of target spending at risk each year (Track 3).

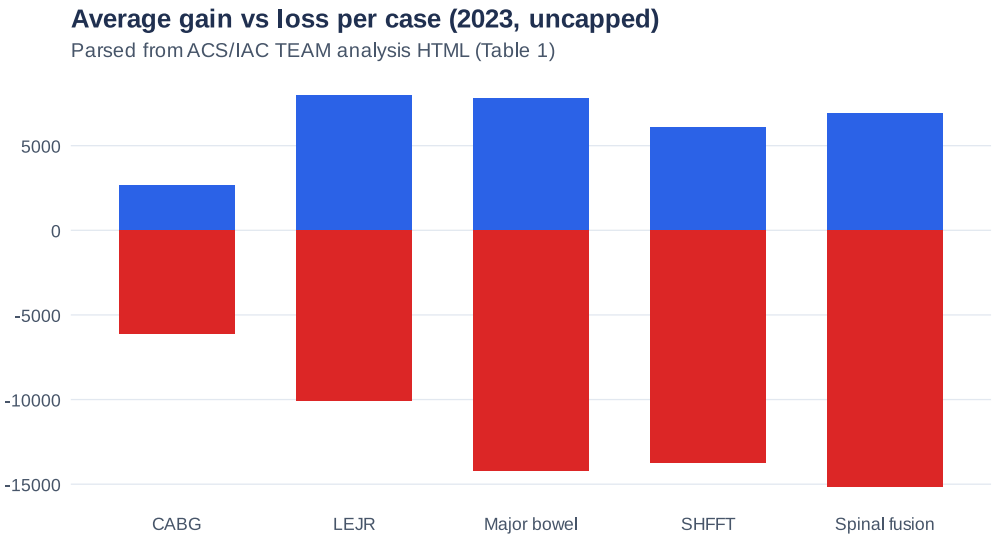


Figure 3: Figure 3. Average gain vs loss per case (ACS/IAC 2023).

Advisory Board modeling: **56%** of mandatory participants projected to lose money under current criteria, averaging **$1.2M** each (13). Five-year exposure above **$14M** is **internal Rainfall analysis** (14).

Penalties & incentives

Track 3 applies a flat **±20%** stop-loss/stop-gain on target price **every performance year** (10) — not a 30% ramp to PY5.

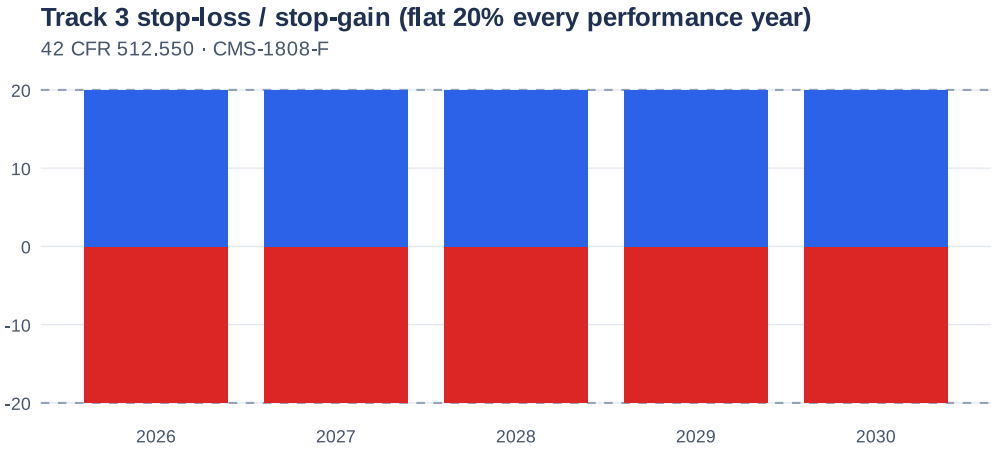


Figure 4: Figure 4. Track 3 corridor (20% each year).

PY1 preliminary target prices span **$16,525** (DRG 331 minimum) to **$134,139** (DRG 426 maximum) across regions (CMS TEAM PY1 workbook).

California

107 mandated hospitals across **11 metro areas** — largest state concentration (10). Region splits: **internal Rainfall analysis** (15).

Episode economics (internal)

Median five-year impact **$2,400,000**; peak **$14,500,000** — **internal Rainfall analysis** (14).

Rainfall Health platform

San Francisco-based Rainfall Health (CEO Eddie Qureshi; **$15M** Series A, Two Bear Capital, February 2026) (16). **$85M** Medicare revenue projection over five years: **internal Rainfall analysis / CEO statements** (16).

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Disclaimer. This brief uses public data, published sources, and internal Rainfall Health analyses (refs 13–14). Not clinical, legal, financial, or regulatory advice.

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